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Phased implementation plan for your India D2C telehealth MVP

Executive summary

Given the earlier research and the direction of your playbook, the best first move is **not** to launch a broad multi-category telehealth brand on day one. The highest-probability path is to launch **adult hair loss** as the first public-facing wedge, while building the underlying engine so it can later support erectile dysfunction, dermatology, diabetes, and eventually PCOS or metabolic care. That gives you the cleanest combination of recurring demand, simple fulfilment, manageable clinical complexity, and lower advertising risk than sexual-health or GLP-1/weight-loss as the initial front door. India's telemedicine rules require patient and doctor identification, consent, defined prescribing pathways, and prohibit AI systems from counselling or prescribing on their own; the drugs-advertising law separately restricts advertising around sexual pleasure, sexual impotence, and obesity-related conditions. [1]

So the right build sequence is:

one acquisition front door → one care workflow → one refill loop → one repeatable paid funnel.

In practice, that means a **web-first MVP** for hair loss, then a controlled alpha, then a low-budget paid beta, then expansion into adjacent verticals only after you have proven prescription-to-delivery reliability, month-two retention, and support quality. The platform can use AI heavily for intake structuring, support triage, ad ideation, and ops automation, but it must not use AI to prescribe or directly counsel patients; that has to remain under a licensed doctor workflow. [2]

Decision

My recommendation is to choose **Hair Loss first**, **ED second**, **Dermatology or Diabetes third**, and hold **GLP-1/weight-loss** for a later phase after clinical governance, fulfilment, and brand trust are already working.

Opportunity	Build now?	Why	Public ad risk	Operational complexity
Hair loss	**Yes**	Good subscription behaviour, discreet need, relatively simple care flow	Lower than ED/GLP-1	Low
Erectile dysfunction	Later	Strong demand, but better as an existing-patient cross-sell first	High	Low
Dermatology	Later	Reuses photo-led async flow	Medium	Low–medium
Diabetes management	Later	Strong retention, but more labs and care coordination	Medium	Medium

Opportunity	Build now?	Why	Public ad risk	Operational complexity
PCOS	Later	Good long-term value, but needs stronger specialist workflows	Medium	Medium
GLP-1 / weight-loss	Not first	Strong demand, but highest clinical, reputational, and ad complexity	High	High
Mental health	Not first	Valuable but requires tighter safeguarding and clinical oversight	High	High

The key legal reason to avoid ED or obesity as the first public acquisition wedge is that the Drugs and Magic Remedies Act prohibits certain kinds of drug advertising relating to sexual pleasure, sexual impotence, and scheduled conditions including obesity. That does not mean you cannot ever operate in these categories; it means you should **not make them your first paid-ad front door**. Hair loss is the cleaner first offer, while ED can sit behind the same onboarding engine once the platform is stable and legal review of copy is complete. [3]

Strategically, I would therefore brand the company **broadly enough to expand**, but make the **hero offer on launch** a single, sharply positioned proposition such as:

“Private, doctor-reviewed online hair-loss treatment for adults in India.”

That keeps the message clear, compliant, and testable.

What to lock before writing code

Before you build anything, there are a few items that must be decided in sequence, because they change what you build.

First, lock the **exact launch scope**: adults only, one condition, one pricing model, one geography cluster, one pharmacy partner, one medical director, and one public message. For the first launch, I would keep it **18+ only**. That materially simplifies consent and avoids the extra burden of verifiable parental consent that applies to children under the data-protection regime. [4]

Second, get a **compliance memo and clinical protocol** done before product design. The product should reflect the rules, not the other way around. The baseline rules from the National Medical Commission [5] telemedicine framework are clear: teleconsults cannot be anonymous, consent must be captured, the prescribing route depends on whether it is a first or follow-up consult and on the mode of consultation, and if a prescription is sent directly to a pharmacy that requires explicit patient consent. The same framework also requires the platform to show the doctor’s name, qualification, and registration number. [6]

Third, lock the **doctor and pharmacy operating model**. Do not build an intake flow before your medical director has approved the inclusion criteria, exclusion criteria, contraindication checks, red-flag escalation rules, and review SLA. Also do not build a dazzling checkout flow before you know exactly how prescriptions will be reviewed, sent, dispensed, and tracked.

Fourth, create a **claims matrix** before you create ads or website copy. Every page headline, FAQ answer, and ad line should be classified as one of the following:

- safe educational claim
- permitted service claim
- risky clinical claim needing evidence/legal review
- prohibited or misleading claim

This matters because your biggest near-term startup risk is not coding speed; it is launching copy that creates advertising or medical-practice exposure. The DMR Act prohibits misleading drug advertisements and is especially sensitive around sexual and obesity-related claims. [7]

Fifth, define the **minimum commercial model** before tech. For the first 60–90 days, you do not need a sophisticated lifetime-value machine. You need one offer that can be sold, delivered, and renewed. I would structure it as:

- **paid initial consult**
- **doctor decision**
- **medication fulfilment**
- **optional monthly follow-up/refill plan**

That is simpler than trying to over-engineer a full continuous-care subscription from day one.

Sixth, define the **event schema** before launch. If you want ad testing to be useful, you need the funnel instrumentation live from day one:

`ad_click → landing_view → quiz_start → quiz_complete → checkout_start → payment_success → consult_completed → rx_issued → pharmacy_order_created → delivered → refill_due → refill_paid`

Without that, you will not know whether a poor result came from ad targeting, the landing page, the form, checkout, doctor response time, or fulfilment.

Seventh, decide what AI is allowed to do. Under the telemedicine framework, AI/ML systems may assist an RMP, but they cannot themselves counsel the patient or prescribe medicines; final counselling and prescription have to be delivered by the doctor. So your AI boundary for MVP should be:

- **allowed:** lead qualification, intake summarisation, support triage, doctor-note drafting, renewal reminders, ops automation
- **not allowed:** diagnosis without doctor review, treatment recommendation shown as a medical decision, automated prescribing, unsupervised clinical counselling [8]

Phased roadmap

The cleanest execution path is a six-phase plan. The logic is simple: do not spend on ads until the clinical loop works; do not expand conditions until the refill loop works; do not build a native app until the web funnel works.

Phase	Timing	Objective	Output	Exit gate
Strategy lock	Week 1	Freeze the first wedge and commercial model	Launch brief, ICP, pricing, claims matrix, legal/ compliance checklist	Founder, medical director, and counsel sign-off
Foundation	Weeks 2–3	Put the regulated rails in place	Entity setup, contracts, doctor onboarding, pharmacy LOI, privacy/ consent draft, analytics map	End-to-end workflow agreed on paper
MVP build	Weeks 4–7	Build the smallest live system	Landing page, eligibility quiz, checkout, doctor dashboard, eRx workflow, notifications, admin panel	Internal team can simulate 10 complete cases
Alpha	Weeks 8–9	Test with controlled real users	20–50 pilot users, support scripts, bug fixes, payment/ refund handling	Prescription-to-delivery works reliably
Beta	Weeks 10–13	Begin measured paid acquisition	Waitlist and live ads, attribution, cohort tracking, retention reminders	CAC and fulfilment metrics are acceptable
Hardening	Months 4–6	Reduce manual work and improve economics	Automation, SOPs, support QA, more content, pricing iteration	Repeatable month-two retention
Expansion	Months 6–12	Add the next vertical	ED cross-sell first, then dermatology or diabetes; later PCOS or GLP-1	New vertical inherits the same core engine

This roadmap is the right order because the underlying legal and operational stack has to exist before live clinical activity. Telemedicine can be delivered through websites, apps, video, audio, and chat-based tools, but the same core compliance duties still apply; meanwhile, the data-protection framework requires you to design consent, security controls, and rights handling into the platform rather than bolt them on afterwards. The 2025 DPDP Rules also have phased commencement, so you should have counsel map which obligations are already live by the time you launch. [9]

How to use the phases in practice

The **first non-negotiable milestone** is not the public website. It is the point at which your team can process one case from start to finish:

lead comes in → patient consents → doctor reviews → prescription issued if appropriate → pharmacy receives it with consent → patient receives order → follow-up is scheduled.

If that is not working internally, do not go live.

The **first paid growth milestone** is not revenue. It is signal clarity. You want to know which message converts, which audience segment completes intake, how many users are ineligible, how long doctors take to respond, and whether delivery issues create refund pressure.

The **first expansion milestone** is not "more SKUs". It is proof that your month-two economics work well enough that adding a second indication will increase value rather than multiply chaos.

Technical configuration

For MVP, the correct product shape is **web-first, mobile-optimised, and workflow-heavy**. Do not start with native iOS/Android apps. The public product should be a fast website with account access and a structured intake flow; the internal product should be a secure doctor-and-ops dashboard. India's telemedicine rules explicitly allow websites, apps, chat platforms, audio, and video as tools, so a web-first launch is fully compatible with the practice model. [10]

Recommended stack

Layer	MVP configuration	Later upgrade
Public frontend	Next.js web app, mobile-first, SSR for landing pages	Add React Native only after product-market fit
Backend	One API service plus background worker queue	Split services later only if load demands it
Database	PostgreSQL	Same, with read replicas later
File storage	Encrypted object storage for images, prescriptions, reports	Add lifecycle rules, archival tiers
Auth	OTP + email/phone login + device/session controls	Add SSO/admin federation later
Video	Secure in-product video provider	Deeper scheduling and virtual waiting room later
Messaging	WhatsApp + email + SMS fallback	Add orchestration and campaign automation later
Payments	Primary gateway + backup gateway + recurring billing	Add tokenised renewals and finance ops tooling
Analytics	Product analytics + ad attribution + server-side event collection	Add BI warehouse later
Admin	Doctor queue, prescription composer, case notes, support actions	Add QA dashboards and workforce planning later
Pharmacy/lab integrations	Manual or semi-manual first, API where feasible	Full partner APIs later
CRM	Light lifecycle automation	Full CRM only once sales and retention flows stabilise

Infrastructure configuration

I would provision the first production environment in India on Amazon Web Services [11] with an India primary region and a separate disaster-recovery posture, because that is the simplest way to keep health-data processing operationally concentrated in India. The DPDP Act applies to digital personal data processed in India and to processing outside India when goods or services are offered to people in India; the Rules also contemplate cross-border transfers subject to central-government requirements. That means your default design should be **India-hosted by default**, with cross-border vendors assessed explicitly rather than casually. [12]

Your production setup should include:

- private network segmentation
- encrypted database
- encrypted file storage
- secrets manager
- full audit logs for doctor actions and prescription events
- role-based access control
- staging environment with synthetic data only
- incident alerting
- nightly backups
- a manual emergency rollback plan

You should also build data-governance features from the start: consent records, privacy notices in plain language, withdrawal handling, erasure workflows, grievance channel, and breach-notification runbooks. The DPDP Act requires notice, valid consent, reasonable security safeguards, and breach notifications to the Board and affected users; if you later become a Significant Data Fiduciary, additional obligations can include a DPO in India, an independent data auditor, and periodic assessments and audits. [13]

Payments and subscriptions

For launch, use Razorpay [14] as the primary gateway and Cashfree [15] as the secondary failover. Recurring renewals should be anchored on the rail supported by the National Payments Corporation of India [16] via UPI AutoPay/e-mandates, because that is the most natural consumer payments path in India for subscription-like use cases. [17]

That said, for the very first beta, it is perfectly reasonable to start with **one-time payment + manual refill renewal** if recurring-payment setup is delaying launch.

Optional ecosystem integrations

Do **not** make national health-stack integration a day-one dependency. Treat Ayushman Bharat Digital Mission [18] integration as a later-stage advantage rather than an MVP requirement. It becomes useful once you have stable records, repeat visits, and a compliance owner who can handle consent-based health-information exchange cleanly. [19]

MVP prompts you can give directly to a website builder

The public website prompt below is designed for a tool such as Lovable, Bolt, Replit, or another AI website/app builder. It intentionally avoids prohibited medical-ad copy, keeps the launch focused, and treats the website as a **conversion + intake + orchestration** layer rather than an autonomous clinical tool. That structure is important because AI cannot directly prescribe or counsel, and service/ad claims must stay within a conservative legal boundary. [20]

Public website prompt

```
Build a responsive, mobile-first web app for an India-based direct-to-
consumer telehealth startup focused on adult hair-loss treatment.

Primary goal:
Convert paid and organic traffic into paid consultations and completed
treatment onboarding.

Brand positioning:
Private, doctor-reviewed online hair-loss treatment for adults in India.
Tone should be trustworthy, clean, modern, discreet, and premium-but-
accessible.
Do not use exaggerated health claims.
Do not say “guaranteed regrowth”, “cure”, “best treatment”, or any
unsubstantiated medical superiority claims.

Audience:
Adults 18+ in India who are experiencing hair thinning or hair loss and want
a discreet online process.

Required pages:
1. Home
2. How it works
3. Pricing
4. FAQ
5. Eligibility quiz
6. Checkout
7. Account / order status
8. Legal pages: Privacy Policy, Terms, Consent Notice, Refund Policy

Homepage requirements:
- Hero section with clear CTA: “Check eligibility”
- Trust section: doctor-reviewed care, discreet delivery, online follow-ups
- 3-step flow: eligibility quiz → doctor review → treatment shipped if
appropriate
- Pricing teaser with starting consultation fee
- FAQ accordion
- Footer with legal links and support contact

Eligibility quiz requirements:
- Mobile-first, multi-step flow
- Ask age, sex, location, hair-loss history, current medicines, allergies,
medical conditions, recent photos upload
- Include red-flag rules and exclusions
- Include explicit consent checkbox in plain English
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- Include checkbox confirming user is 18+
- Save progress
- Pass structured output to admin dashboard

Checkout requirements:
- Consultation fee payment
- UTM capture from traffic sources
- Coupon code support
- Thank-you page with next steps and expected turnaround time

Account area requirements:
- Login via OTP
- Show consultation status
- Show messages from support
- Show prescription/order status
- Show refill reminder card if applicable

Compliance and UX rules:
- Display a clear statement that doctor review is required before any prescription
- Display “not for emergencies”
- Do not use fake doctor images or fake credentials
- Add placeholders for real doctor profile, qualification, and registration number
- Add privacy consent notice in clear language, separate from Terms
- Add support channel for grievances and privacy requests
- Do not present AI as diagnosing or prescribing

Tracking:
Implement analytics events for:
landing_view
cta_click
quiz_start
quiz_complete
checkout_start
payment_success
thank_you_view
account_signup
support_contact

Design requirements:
- Fast loading on Indian mobile networks
- SSR for SEO pages
- Clean layouts, large tap targets, simple typography
- English-first content with architecture that allows Hinglish later
- No clutter, no marketplaces, no broad health catalogue

Tech preferences:
- Next.js
- TypeScript
- PostgreSQL

```
- Secure file upload for patient photos
- Admin-ready architecture
- Environment variables for payment gateway, analytics, messaging, and API
endpoints
```

Internal doctor-and-ops prompt

```
Build a secure internal admin dashboard for the same telehealth startup.

Users:
- Doctor
- Medical director
- Operations/admin
- Customer support

Required modules:
1. Case queue with filters: new / in review / approved / declined / follow-
up / urgent
2. Patient profile with submitted quiz answers, uploaded photos, interaction
history
3. Structured clinical notes template
4. Prescription composer with approval workflow
5. Messaging panel for non-emergency patient communication
6. Order panel to mark prescription sent to pharmacy, order created, shipped,
delivered
7. Follow-up scheduler and refill reminder engine
8. Audit log for every case action
9. Access control by role
10. Dashboard for daily metrics

Compliance behaviour:
- No auto-prescribe flow
- Every clinical action must be attributable to a real doctor
- Show doctor identity and registration number in admin records
- Require consent status before any prescription transmission
- Log every change to prescription or patient status

Ops features:
- Flag ineligible patients automatically
- Flag missing photo uploads
- Flag orders stuck in fulfilment
- Export daily queue report
- Support tags: refund, delay, side effect, privacy request, cancellation

Analytics:
Track time-to-review, approval rate, prescription rate, delivery completion,
refund rate, refill conversion, and support ticket themes.

Tech:
- Same codebase if possible
```

```
- Protected routes
- RBAC
- Encrypted storage
- API-first design
```

Paid acquisition and learning loop

You should not go from "build complete" straight to "full-scale paid ads". Do it in three steps.

Mock stage

Before any live clinical traffic, launch a **message-market-fit landing page** and run:

- internal user testing
- friend-and-family beta
- unlisted/draft ad creative review
- organic and direct traffic tests
- optional waitlist capture

The purpose here is not to generate revenue. It is to test the message, the form, the conversion flow, and the support scripts without exposing a broken ops loop to the market.

Controlled paid beta

Once the prescription-to-delivery loop works, begin with a **small live ad budget** and one offer. I would start with:

- high-intent search ads
- a small Meta creative test
- retargeting only after enough visits exist

The website should send all paid traffic to one of only two flows:

- **"check eligibility"**
- **"book paid consult"**

Do not send cold traffic to a broad homepage with too many options.

Because the ad law is sensitive around misleading claims and certain medical-condition advertising, your first ad creatives should be **service-led**, not miracle-led. Good examples for hair-loss launch are:

- "Private online hair-loss consult from home"
- "Doctor-reviewed treatment plans for adults"
- "Discreet delivery across India"
- "Start with an eligibility check"

Avoid before/after claims, "guaranteed results", competitive-superiority claims, and any copy that crosses into prohibited medical advertising. [7]

Scaling stage

Only scale budget when these internal thresholds are healthy for at least a few consecutive weeks:

- landing page conversion is stable
- doctor turnaround time is acceptable
- fulfilment failures are rare
- support tickets are manageable
- month-two retention is visible
- refunds are not dominating paid customer cohorts

At that point, you can increase spend, improve creative breadth, add UTM-driven lifecycle messaging, and turn on subscription renewals more aggressively. The recurring-payment setup should lean on UPI AutoPay support through your gateway, but the commercial logic still needs human monitoring for failed mandates, dunning, and cancellation reasons. [17]

A practical working funnel for the first paid beta is:

Step	What you test	What success tells you
Ad click-through	Message appeal	Are people interested in the promise?
Quiz start rate	Landing clarity	Is the hero offer understandable?
Quiz completion	Intake UX	Is the form too long or too intrusive?
Payment conversion	Offer/value trust	Will users pay for the first consult?
Consult completion	Ops quality	Are scheduling and reminders working?
Prescription/eligibility rate	ICP quality	Are ads attracting the right users?
Delivery completion	Partner reliability	Can you operationally fulfil at low friction?
Refill conversion	Product-market fit	Do users trust the service enough to continue?

Recommended execution order from here

If I were running this build, I would use this exact order:

adult hair loss website first → alpha patients → controlled paid beta → refill system → ED as cross-sell for existing patients → dermatology or diabetes as the next public vertical → PCOS after specialist ops are ready → GLP-1 only after clinical governance and supply are proven.

That sequence aligns the product with the real constraints set by telemedicine rules, data-protection obligations, AI boundaries, and Indian advertising law, while still giving you a fast path to something you can actually launch, learn from, and scale. [21]

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[1] [2] [6] [8] [9] [10] [11] [20] [21] https://nmcn.in/public/assets/pdf/Telemedicine%20Practice%20Guidelines.pdf
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